



Policy

Transportation of Mental Health Consumers

Policy code	EMHS: 20
Original endorsement date	July 2016
Date of current endorsement	October 2022
Version	2.0
Functional subgroup	Mental Health
Summary	Transporting mental health patients must be undertaken in a manner that is least restrictive to the patient and maintains the safety of both the patient and escorting staff /others in accordance with Department of Health policy MP0063/17
Policy owner	Executive Director Safety Quality and Consumer Engagement
Contact	
Review date	October 2025
NSQHS Standards (second edition)	2010. 11.3.6, 11.3.8,
Status	Active

1. Background

The purpose of this policy is to outline the principles and procedures for mental health clinicians in making decisions regarding transport of patients with a mental illness required facilitate the patient's assessment, examination and/or treatment.

The decision to transport a patient, including in a government vehicle/ambulance/private vehicle/taxi is a clinical decision and must be documented in the patient's medical record. The clinician arranging transport and/ or transporting/ escorting the patient is responsible for ensuring a risk assessment and risk management plan has been completed and present in the patient's health record prior to the transportation. Where the risk assessment and risk management plan already exists in the patient's medical record they are to be reviewed and documented in the integrated progress notes prior to the transport occurring.

Refer to [MP0099/18 Community Mental Health Status Assessment: Role of MH Clinician](#)

1.1. Principles

- An objective of the *Mental Health Act 2014* (MHA 2014) is to ensure that people with a mental illness are provided with the least possible restriction of their freedom and the least possible interference with their rights.
- Patients have the right to safe transport that minimises restriction to their rights, dignity and self-respect, and that reduces the likelihood that they will experience transport that is a traumatic event. This right needs to be balanced with the safety of all concerned and the active management of risk. Any restriction of a person's rights must be reasonable and proportional.
- Transportation not only enables access to mental health care but is also a point of care provision itself.
- Transport carries potential risks; an appropriate level of consideration and collaboration to ensure safety for all involved must be undertaken.
- Patient's should be encouraged to make their own decisions and arrangements for transport wherever possible and when clinically and legally appropriate.
- When a patient is under a MHA 2014 Form 1A, Form 3D, Form 4C, Form 5F, Form 6A, Form 6B or Form 7C the medical practitioner or authorised mental health practitioner should consider the safest and least restrictive means of transportation that is reasonably available.
- In determining the mode of transport to be used, consideration should be given to the current situation, previous clinical alerts, relevant risks and impact on the patient and/or their family, personal support persons (PSP) /carers.

1.2. Specific Considerations

To determine the most appropriate mode of patient transport mental health clinicians must consider the following:

- the patient's legal status
- current and past mental health history and presentation
- current and past risk assessment(s), including alerts

- the patient's physical health
- the patient's immediate treatment needs
- the risk of harm the patient poses to themselves and/or others
- the distance to be travelled, available modes of transport and the likely effect on the patient of the proposed mode of transport
- the patient's need for clinical support, supervision and/or sedation during the period of travel (as appropriate)
- information from other service providers, family/PSP/ or carers and
- whether there has been any reference to transport preferences as part of a patient's advance healthcare directive.

Time spent addressing particular clinical or psychosocial needs before transportation may allow a safer or less distressing transport experience. Where potential delays for transport may occur, this should be discussed with the clinical team. Some examples that may require consideration prior to transport include:

- a patient waiting to see a medical practitioner for a risk assessment before moving to another hospital
- a patient wanting to secure their property or animals before being taken to hospital
- delaying transport until family members/PSP/carers are present to support the patient.

2. Transport Options

Safe transportation requires meeting the clinical care needs of the patient and is underpinned by safety considerations, the principle of least restrictive care and risk management. In the event a clinician is unable to decide the most appropriate transport option, this should be discussed with their clinical supervisor or the multidisciplinary team (MDT) for decision.

Refer to System Manager policy MP0063/17

Refer to EMHS [Care Coordination in Mental Health Policy](#)

Refer to [Standard 4 Chief Psychiatrist physical health care of Mental Health Consumers](#)

2.1. Transport by private vehicle or taxi

A private vehicle driven by a family member/personal support person (PSP)/carer, or a taxi with a patient travelling either alone or accompanied by a PSP/ or family/carers may provide the most accessible and acceptable mode of transport. When deciding whether to transport a patient in a private vehicle or taxi, staff must consider:

- the patient's current mental state, especially the risk of unpredictable or erratic behaviour
- the patient's understanding and acceptance of the purpose and destination of the transport
- the driver's understanding of the purpose of and the destination of the transport;
- the distance to be travelled

- patient's history of using this mode of transport; and
- safety or risk issues that may impact the driver or other passengers in the vehicle.

Staff should not try to persuade a family member /PSP/carer to transport a patient if they are unwilling.

2.2. Transport by mental health service vehicle

Where the patient is known to the treatment team, and there is no clinical or safety requirement for an ambulance, the use of a government vehicle to transport a patient from the community to an emergency department or inpatient service may be appropriate.

When deciding whether to use a government vehicle for transport, staff must consider:

- the patient's current mental state, especially their risk of unpredictable or erratic behaviour
- the patient's previous transport history
- the patient's understanding and acceptance of the purpose and destination of the transport
- the distance to be travelled and the time of day
- the escorting clinician's knowledge of the patient and their history (e.g. the clinician is to be made aware of the patient and their history);
- placement of the patient in the vehicle should be in the rear seat next to a clinician, not behind the driver
- the patient's willingness to be transported by the proposed driver
- the safety needs of all persons in the vehicle.

When transporting a patient in a government vehicle an accompanying staff member in addition to the driver should be considered in case there is a change in patient circumstances or there is a need to contact others for assistance.

2.3. Transport by Ambulance

Wilson Medic One, St John Ambulance Australia and National Patient Transport have primary responsibility for the transport of people with a mental illness who need treatment in hospital and are to be transported by clinical staff alone. The following three contracts between Department of Health WA and Transport contractors/providers:

- C06423 Emergency Ambulance Services
- HCNS 120214 Non- Emergency Inter-Hospital Patient Transport
- Mental Health Transport Services

The Emergency Ambulance Services contact is the 000 service provider. An emergency ambulance is used when a person has an urgent need for medical treatment for a physical or mental illness and where there is an urgent need for observation and clinical supervision of the health and wellbeing of a patient.

The providers for these services are:

Emergency – 000 – any referrer

- St John Ambulance Australia

Non-Emergency Inter-Hospital Patient Transport (hospital referrals including Emergency Departments (ED's) St John Ambulance Australia

- Wilson Medic One
- National Transport Services

Mental Health Transport Services (patients under the MHA, 2014) – (community and hospital)

- Wilson Medic One

These contracts are expiring on 1 July 2021 and the Department of Health are responsible for progressing these contracts.

Also see health point page [Mental Health Transport](#) service contacts

3. Accessing Ambulance Services

When requesting an ambulance, staff should be prepared to provide relevant information, including:

- the current location and planned destination of the patient requiring transport
- the patient's age, diagnosis, current medical condition and mental state
- the patient's current medication regimen and whether there is any evidence of recent medication abuse
- whether the patient is intoxicated, in withdrawal or experiencing any other substance-related problems
- the legal status of the patient (voluntary or involuntary) and the readiness of any associated documentation (legal orders that are required to be provided to the ambulance, transport officer or police officer are [Form 4A](#) Transport Orders and Form 7D Apprehension and Return Orders only)
- any safety issues or alerts that would help prioritise the call appropriately
- the availability of clinical staff and whether the patient is known to those clinicians
- the name of the person or service who will be receiving the patient
- whether the patient has the potential for aggression /violence or requires restraint as a last resort
- whether or not police assistance is required
- a description of the situation, who is present and the presentation of the patient. Paramedics will determine if it is safe to approach the scene,
- whether the patient has been sedated
- the contact number for the most appropriate person to advise about the patient,

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should a call back be required.

3.1. Attending

The clinician requesting the ambulance should be present when the ambulance arrives to ensure:

- the person being transported can be identified correctly and introduced to the transporting team
- clinical handover and appropriate documentation is provided
- the person to be transported is not left unaccompanied
- any necessary communication or debriefing with family/PSP/ or carers can occur.

Exceptions to this include for emergency situations, for example include a suspected drug overdose that necessitates a request for an ambulance that arrives before mental health staff can attend the patient.

3.2. Escorting

Mental health professionals are not required to accompany the patient during transportation by ambulance, however some patients will require an escort to travel with them to ensure adequate support. An escort may be a family member, PSP/carer, a mental health professional or a police officer. This will be determined by the patient's presentation, needs and risks.

3.3. Accompanying mental health professionals

Prior to transport commencing

If a mental health professional accompanies the patient their role requires specific discussion and will depend on the needs of the patient being transported and the type of transport involved. The mental health professionals should:

- assist in preparing the patient for transport including items they may require for inpatient admission
- support and reassure the patient
- ensure members of the treating or receiving mental health teams are aware of the transfer and transport plans
- ensure the relevant mental health and statutory transport authorisation documentation has been completed (see section on legal framework)
- provide the patient and carer/s with information about the transport arrangements
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3.4. Clinical Handover

A detailed clinical handover between the attending mental health professional and transport officers should include:

- briefing on the patient's physical and mental state
- relevant personal details and next of kin contact details
- details of arrangements made for any dependent children or animals

- details regarding any sedation administered and potential need for restraint
- risk assessment
- transport and inpatient admission requirements
- the patient's legal status
- the nature of any documentation that will accompany the patient
- the name of any receiving clinician or service expecting the patient.

The best form of communication in these circumstances is usually face-to-face discussion using the ISoBAR format between the various professionals at the scene as per [Clinical Handover policy MP0095](#).

3.5. Transport by police

Transport of a person with a mental health illness by police should only be considered when all other transport options have been deemed unsuitable.

Police may be requested to attend when there is a significant risk of harm to the patient or others during transportation. Transport in a police vehicle is more restrictive and may add to stigma and/or trauma for the patient and family /PSP/carer.

In some situations, ambulance paramedics may determine they cannot provide transport without police assistance. This decision must be made between mental health clinicians and ambulance officers prior to contacting police.

3.5.1. Transport by Police Process:

1. Completion of a comprehensive Mental Health assessment and risk assessment and management plan
2. Determination of *MHA, 2014* [requirements \(which MHA, 2014\) forms are in place](#)
3. Completion of Transport Order Form 4A (MHA, 2014) marking Officer Responsible for carrying out this order: ☐ "Police officer" - [see exemplar](#):
4. Completion of [Mental Health Transport Risk Matrix Form](#) – indicating applicable "Significant Risk" elements in the matrix
5. Call Police 131 444 and log Police escort request for a Mental Health Patient and obtain the police Computer Aided Dispatch (CAD) number
6. Write the CAD number on the front of the Mental Health Transport Risk Matrix Form and send via email the:
 - Form 4A (Transport Order) (or Apprehension and Return Order – 7D if this is applicable)
 - Mental Health Transport Risk Matrix Form
 - Ensure your primary and secondary contact details are included on the paperwork
 - Send To: Communications.Mental.Health.Smail@police.wa.gov.au.

No further MHA, 2014 legal forms or information are required beyond what is stipulated above.

Refer to MP0063/17 [Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders](#)

4. Documentation

The attending mental health clinician must ensure all relevant documentation is completed.

Rationale underpinning the decision to transport a mental health patient must be documented in full in the patient's integrated progress notes. This must include completion/ consideration of risk assessment and risk management plan and previous clinical alerts.

Escort arrangements are to be documented in the appropriate forms / notes.

Refer to EMHS Rights of Patients in Mental Health Policy.

[A Transport Risk Matrix Form](#) must be completed and accompany the Mental Health Act (2014) Form 4A when requesting WA Police assistance for transfers.

5. Compliance monitoring and legislative penalties

5.1. Compliance monitoring

Each EMHS Site/ Service Executive Director is to ensure compliance with this policy.

5.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

6. Legislation

- *Mental Health Act 2014* sections 29, 63, 67, 92, 112, 129, 133, 148, 150
- Mental Health Code of Practice Standard 2015

7. Additional references

7.1. Department of Health policies

- [MP 0012/16 Missing Persons Policy](#)
- [MP0063/17 Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders Policy](#)
- MP0095 Clinical Handover Policy
- MP0099/18 Community Mental Health Status Assessment: Role of MH Clinician

7.2. EMHS policies

- [EMHS Missing or Suspected Missing Mental Health Patients/Consumers Policy](#)
- [Rights of Patients in Mental Health Policy](#)
- [Care Coordination in Mental Health Policy](#)
- healthpoint page [Mental Health Transport](#) service